ICCE | The International Center for Clinical Excellence

Feedback-Informed Treatment GAP Assessment Tool (FITGAP)

Realm 1: CLINICAL

(Version 2.0)

Not Applicable	No, None, Never	Very Limited, Not Often	Partially. Frequently	Mostly, Regularly	Yes, Fully Always
N/A	1	2	3	4	5

Write in the score that best applies

Helpers:

1. Administer and score the Outcome Rating Scale (ORS) and Session Rating Scale (SRS) together with the client (and/or collateral rater) at each visit or "unit of service." ☐ Score

2. Compare client score on the ORS to the clinical cutoff and discuss with the client at the first visit. ☐ Score

3. Compare client scores on the ORS at each visit to the "treatment response trajectory" (TRT) to determine which clients are making progress and which are at risk for a negative or null outcome. ☐ Score

4. Discuss client progress (ORS) and experience of the relationship (SRS) at each visit. ☐ Score

5. Use the ORS and SRS to develop and refine individualized treatment planning. ☐ Score

6. Use ORS and SRS data to determine which cases need to be discussed in FIT consultation. ☐ Score

Consultation & Supervision:

7. FIT consultants have access to individual helper and program outcome data to facilitate the identification of "at risk" clients. ☐ Score

8. Access to FIT consultation is sufficient to address the number of cases identified as "at risk" for a negative or null outcome. ☐ Score

9. Any supervision beyond FIT consultation is consistent with and friendly toward FIT principles and practice. ☐ Score

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Write in the score that best applies

10. FIT consultation and other supervision provided encourage flexibility and diversity in methods, approach, and providers in order to accommodate individual client culture, preferences, and worldview. ☐ Score

Training on FIT:

11. Is based on a structured curriculum and training plan consistent with the "Core Competencies of Feedback-informed Treatment." ☐ Score

12. Is available on an ongoing basis to both new and existing staff (i.e., helpers, managers, supervisors). ☐ Score

13. Is offered onsite by "in house" FIT experts/champions ☐ Score

Client/Service Users:

14. A description of and rationale for FIT (including the process and timeframe for dealing with unhelpful or undesired services) is provided at first contact. ☐ Score

15. Clinical records (e.g., assessments/evaluations, correspondence, FIT SDA, progress notes) are completed in an open and collaborative manner. ☐ Score

16. FIT "Service Delivery Agreement" (SDA) is organized around their priorities, goals, preferences, and progress. ☐ Score

17. Level, intensity, and type of service(s) offered are informed by their initial score on the ORS. ☐ Score

18. Feedback via the ORS and SRS is taken seriously and used on an ongoing basis to inform and adjust service(s). ☐ Score

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Realm 2: ADMINISTRATIVE

(Version 2.0)

Not Applicable	No, None, Never	Very Limited, Not Often	Partially. Frequently	Mostly, Regularly	Yes, Fully Always
N/A	1	2	3	4	5

Write in the score that best applies

The Agency

19. Client/service user outcomes are a central feature of the agency's "Mission Statement" and strategic plan. ☐ Score

20. Agency has policies and practice guidelines consistent with the principles and practice of FIT including, but not limited to:

    a. How outcome and relationship data are collected, utilized, and shared with clients, staff, managers, and external stakeholders. ☐ Score

    b. Using individual client outcome data (i.e., SPI [Success Probability Index] and TRT) to guide care (e.g., intensity, type, provider) and discharge planning. ☐ Score

    c. Timing and process of ending services or transferring clients to different providers or service settings when outcome data indicate care is unhelpful. ☐ Score

    d. Using outcome data to guide the type and funding of professional development activities. ☐ Score

21. Knowledge about, attitude toward, and experience using FIT are essential qualifications in the hiring of new staff and leadership ☐ Score

Management and Leadership

22. Leadership and managers are knowledgeable about FIT and follow the evidence-based steps of implementation ☐ Score

23. Leadership and managers actively lead the implementation of FIT (e.g., attending the Transition Oversight Group [TOG], establishing an implementation plan and budget, addressing barriers, and establishing an accountability framework) ☐ Score

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Realm 2: ADMINISTRATIVE

(Version 2.0)

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N/A	1	2	3	4	5

Write in the score that best applies

24. Leadership and managers foster a "culture of feedback" at the agency (e.g., being open and transparent, interested in and receptive to feedback from staff, supervisors, and service users) ☐ Score

25. Leadership and managers ensure client feedback regarding progress and quality of the relationship is included in all clinical discussions ☐ Score

26. Leadership and managers have secured support for FIT from the governing authorities (e.g., Board of Directors, funders, consumer organization, regulatory and other oversight bodies) ☐ Score

27. Leadership and managers ensure agency personnel have the time available in their schedules to meaningfully and effectively utilize FIT. ☐ Score

Support Personnel

28. Administrative staff (receptionists, administrative assistants, custodial and other non-clinical employees) have been trained in the principles of FIT ☐ Score

29. Administrative staff (receptionists, administrative assistants, custodial and other non-clinical employees) embody the "culture of feedback" in their interactions with service users (e.g., are open and transparent, interested in and receptive to feedback). ☐ Score

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Feedback-Informed Treatment GAP Assessment Tool (FITGAP)

Realm 3: DOCUMENTATION & INFORMATION TECHNOLOGY (IT)

(Version 2.0)

Not Applicable	No, None, Never	Very Limited, Not Often	Partially. Frequently	Mostly, Regularly	Yes, Fully Always
N/A	1	2	3	4	5

Write in the score that best applies

Documentation and IT

30. One of the three, authorized FIT outcome management systems has been adopted and is being used in all programs and service settings (e.g., inpatient, outpatient, in home, outreach, individual/couple/family, psychiatric and group). ☐ Score

31. A specific staff member has been assigned to oversee and manage access to and organization of the outcome management system (e.g., subscriptions, licenses, and hardware purchases). ☐ Score

32. Staff (e.g., helpers, supervisors, managers) are skilled in using the outcome management system. ☐ Score

33. Support and training on the use of the outcome management system is easily accessible and available on an ongoing basis. ☐ Score

34. Clinical documentation is consistent with and supportive of FIT principles and practice (i.e., FIT service delivery agreement and progress note). ☐ Score

35. Clinical documentation (e.g., administration of the measures, development of the FIT SDA and progress note) is completed in a collaborative manner, together with clients. ☐ Score

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Realm 4: STAKEHOLDERS

(Version 2.0)

Not Applicable	No, None, Never	Very Limited, Not Often	Partially. Frequently	Mostly, Regularly	Yes, Fully Always
N/A	1	2	3	4	5

Write in the score that best applies

Legal, regulatory, accreditation, and funding

36. Managers and the TOG have identified and addressed potential conflicts between the principles and practice of FIT and legal mandates, accreditation standards, and funding/funder requirements. ☐ Score

37. Managers and the TOG have sought variances from legal, accreditation and funding bodies for mandates or standards representing significant barriers to FIT implementation ☐ Score

Referral sources

38. Managers and the TOG have informed referral sources about:

    a. The use of FIT to determine the level and intensity of care. ☐ Score

    b. Their potential role as "collateral raters." ☐ Score

    c. The formal process and time frame for determining dealing with care that is unhelpful or undesired. ☐ Score

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